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Richard C. Lotsch, D. O.
EMS Agency Medical Director

POLICY MEMORANDUM 6607

DATE: 20 November 2008

REVIEWED/APPROVED BY:

RICHARD C. LOTSCH, D.O., EMS AGENCY MEDICAL DIRECTOR

MICHAEL A. FRENN, EMS AGENCY ADMINISTRATOR

SUBJECT: TASERED PATIENTS AND THE REMOVAL OF BARBS

AUTHORITY: CALIFORNIA HEALTH & SAFETY CODE, §1797.204, §1797.220

I. PURPOSE/POLICY:

To establish guidelines for paramedics in the treatment of a tasered patient and the possible removal of the taser barbs from the patient.

II. GENERAL CONSIDERATIONS:

A. Law Enforcement has several options to use when controlling a violent or combative subject including the use of a Taser. A Taser is designed to transmit electrical impulses that temporarily disrupt the body's central nervous system. Its Electro-Muscular Disruption (EMD) technology causes an uncontrollable contraction of the muscle tissue, allowing the Taser to physically debilitate a target regardless of pain tolerance or mental focus.

B. The scene must be safe and secured by law enforcement before Emergency Medical Services (EMS) will evaluate or treat the patient.

C. Assess the patient for any potential cause of the abnormal or combative behavior such as but not limited to hypoglycemia, hypoxia, psychosis or intoxication and treat per the appropriate protocol.

D. Assess the patient for any potential injury after the Taser was deployed. Remember the Taser may cause the patient to fall to the ground.

E. A careful medical history should be obtained, including but not limited to cardiac history and substance abuse problems including stimulant or cocaine use. Thought should be given to the condition of agitated delirium and treat if necessary. Follow Policy 6143, Patient Restraint and use of sedating drugs.

III. PROCEDURE:

A. Remove Taser darts only if they in a location which could interfere with safe, secure patient packaging for transport. **Removal of the taser darts does not constitute medical clearance of the patient. Every patient who has taser darts removed by EMS must be transported to a hospital ED for medical evaluation.** If the police want EMS to remove the darts, then they must agree to have the patient taken to a hospital. If police do not agree with that, then EMS will not remove the darts.

B. Never remove taser darts from the eye, neck or groin.

C. Before touching any patient who has been tasered, insure that the police officer has disconnected the wires from handheld unit or cut the wires.

D. Utilize PPE. Place hand in the form of a “V” around the taser dart in order to stabilize the surrounding skin and to keep loose skin from coming up with the dart. Firmly grasp the dart and with one smooth hard jerk, remove the dart from the patient’s skin.

E. Prior to dart removal inform all caregivers that you are about to remove the contaminated sharp.

F. If the barbs do not come out with a firm tug; then leave them in place and have the ER remove the barbs. The darts may have penetrated deeper into the dermis.

G. Visually inspect the dart to make sure the tip did not break off during removal and dispose of the sharp in the appropriate container.

H. Cleanse the wound with saline and apply a covering over the area.

IV. PCR DOCUMENTATION:

The following must be documented on the PCR:

A. The patients’ presenting behavior or signs and symptoms which lead law enforcement to tase the patient.

B. Baseline patient assessment including but not limited to Oxygen Saturation; Blood Glucose level, neurologic assessment including pupil assessment, vital signs including skin signs and Level of Consciousness and repeat patient assessment at least every 10 minutes until arrival at the emergency department.

C. Time of Removal.

D. Anatomic location of the darts.

E. Whether or not taser darts are intact after removal.

V. QUALITY IMPROVEMENT:

A. Each ALS department and their Medical Director annually will review this policy as will all personnel and provide documentation of training when asked by the EMS Agency.

B. Each ALS department will annually conduct a random audit of PCRs to evaluate the thoroughness of documentation for each case.

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Ted Selby
Emergency Services Bureau Chief

POLICY MEMORANDUM 6609

DATE: September 1, 2013

REVIEWED/APPROVED BY:

AARON BAIR, MD, MSc., EMS AGENCY MEDICAL DIRECTOR

TED SELBY, EMS AGENCY ADMINISTRATOR

SUBJECT: ST – Segment Elevation Myocardial Infarction (STEMI)

AUTHORITY: CALIFORNIA HEALTH & SAFETY CODE, DIVISION 2.5, § 1797.220

PURPOSE/POLICY:
This policy is intended to provide paramedics guidance on the treatment and transport of patients suffering from a ST - Segment Elevation Myocardial Infarction (STEMI). The purpose of this policy is to decrease morbidity and mortality. This can be accomplished by patient education and early public recognition of cardiac problems, a systematic approach by EMS, and a coordinated effort by local hospitals to intervene with percutaneous coronary intervention (PCI). This policy establishes a system-wide policy for the care of patients with this life threatening cardiac illness.

I. DEFINITIONS

- **Acute Coronary Syndrome (ACS) -** This is an umbrella term used to cover any group of clinical symptoms compatible with acute myocardial ischemia.
- **Percutaneous Coronary Intervention (PCI)** - otherwise known as angioplasty is the use of a small flexible balloon catheter to open a blockage in a coronary artery.
- **Reperfusion** – Re-establishing blood flow to cardiac tissue after suffering from a coronary artery occlusion.
- **ST Segment Elevation Myocardial Infarction (STEMI).**

- **STEMI Receiving Center (SRC)**– A hospital designated by the Local EMS Agency (LEMSA) that is capable of appropriately treating a patient having a STEMI with PCI and other interventional cardiology procedures to restore circulation to a blocked artery.
- **STEMI Referral Facility (SRF) –** A system hospital that refers patients' with a STEMI to a SRC for advanced therapeutic intervention such as, but not limited to, PCI.
- **Thrombolytic** – Medication used to dissolve blood clots in the body.

II. FIELD CARE

A. Paramedics evaluate patients and provide treatment according to protocol.

B. Paramedics (First Responder and/or Transport) will complete a 12 lead EKG as soon as possible.

C. If the 12 Lead EKG reading shows ***Suspected Acute MI***, or Solano County authorized equivalent, the paramedic will follow Protocol C 14.

D. For unstable patients, contact base hospital physician and consider transport to closest facility.

E. ALS Providers will notify the EMS Agency in writing within seven (7) days of purchasing or updating 12-lead ECG software or a new 12 lead monitor.

III. DESTINATION

A. If field 12 Lead EKG shows ***Suspected Acute MI***, or Solano County authorized equivalent, the paramedic will use protocol C14 to make a hospital destination decision.

1. Patients in cardiac arrest with Return of Spontaneous Circulation (ROSC) will be transported to the nearest authorized SRC.

B. Paramedics will leave the 12 Lead EKG electrodes in place on the patient's chest arms and legs so the hospital can quickly obtain their own 12 lead EKG.

C. Based on geographic location the paramedic will transport patient to the designated Solano County SRC that can be reached in the shortest length of time.

D. The LEMSA will identify in and out-of-county STEMI Receiving Centers authorized to receive transports from Solano County paramedics.

IV. PCI CENTERS/STEMI Receiving Centers (SRC)

A. Any Hospital can become a PCI Center or STEMI Receiving Center providing they participate in a national data registry and meet the following criteria:

1. Complete Solano County EMS Agency's SRC application and pay the appropriate annual fee of $10,000.00 (Ten Thousand dollars);
2. Sign the appropriate written agreement, agree to participate in Solano County data collection, and attend appropriate county meetings;
3. Data must be entered in the following data registries: National Cardiac Data Registry (NCDR), CathPCI, and ACTION;

4. Must meet the current SRC criteria of the American College of Cardiology and the American Heart Association.
5. Must appoint co-medical directors for the SRC:
   a. An emergency medicine physician and
   b. An interventional cardiologist,

      or

      A physician who has completed a fellowship in interventional cardiology and preparing to take boards,

      or

      A physician who meets the following criteria:
      i. Completed post graduate cardiology training prior to July 1, 2002, and;
      ii. Has at least seven to ten years of primary operator experience in therapeutic PCI, and
      iii. Has current hospital staff privileges in at least one hospital for the past five years, and
      iv. Has completed at least 150 therapeutic cardiac catheterizations in the past 2 years.
   c. The EMS Medical Director may consider granting a waiver to physicians who may not meet the criteria listed above on a case by case basis.
6. Must also appoint an SRC Program Manager or SRC Program Coordinator.
7. Must undergo a Pass/Fail SRC Site Survey by a committee, which may include, but is not limited to: Solano County EMS Agency Medical Director, Solano County EMS Agency Administrator, Local EMS Agency Quality Assurance Nurse, and a subject matter expert with experience in designation and oversight of STEMI Receiving Centers in California. The STEMI Receiving Center Site Visit and Evaluation Tool will be provided upon request, or upon receipt of the Application for Designation as a Solano County Approved STEMI Receiving Center. The cost of the site survey will be paid for by the applicant.
8. Must provide Solano County EMS Agency appropriately formatted data in a timely manner (see attachment A).
9. Must receive any patient designated as a “STEMI patient” from the field and provide the appropriate level of care based on the patient’s condition.
   (a) Patient’s suffering from cardiac arrest with ROSC will be transported to the closest SRC for the appropriate clinical therapy (e.g. hypothermia, cardiac catheterization, etc.).

V. QUALITY IMPROVEMENT

A. ALS FIELD PROVIDERS

1. Monthly (by the 10th of the month, i.e. March data due April10th) provide the EMS Agency PCRs and 12 lead EKGs of those patients who had STEMI activation.
2. Monthly (by the 10th of the month, i.e. March data due April10th) provide a list of all patients treated with the C-14 protocol.
3. Quality Improvement (QI) representative will attend the Solano County STEMI Review Meeting.
4. Comply with any policy or protocol concerning STEMI.
5. Provide system paramedics with four hours of annual training on Acute Coronary Syndrome care, 12-lead EKG and associated protocols and policies. (This will be verified at the time of each paramedic's Solano County reaccreditation cycle.)

B. STEMI Referral Facilities

1. Provide data to the EMS Agency on the number of patients treated in their ED for STEMI, the mode of arrival, and whether the patient was treated with thrombolytics or transferred for primary PCI.
2. Send representative to the Solano County STEMI QI Meetings.
3. Adopt a plan to treat STEMI patients with either thrombolytics or transfer for primary PCI.

C. SOLANO COUNTY DESIGNATED STEMI RECEVING CENTERS - SRCs

1. Within 10 days of patient arrival complete the Solano County EMS STEMI report and forward to the EMS Agency. Data elements include, but are not limited to:
   a. Time STEMI Alert called by EMS;
   b. ED EKG STEMI;
   c. ED Arrival Time and date;
   d. Intervention done;
   e. Intervention date and time;
   f. Summary of cardiac cath findings.
2. Provide properly formatted data quarterly to the EMS Agency. STEMI Data elements include:
   a. Number of STEMI Alerts;
   b. Number of confirmed STEMIs (of those with alert);
   c. Number of Interventions and Type (PCI or thrombolysis);
   d. Door-to-Intervention (median) by type (in minutes);

e. Percentile of Door-to-Intervention 90 minutes or less (PCI), 30 minutes or less (thrombolytics).

3. Provide properly formatted data annually to the EMS Agency. STEMI Data elements include, but are not limited to:

   a. EMS Data Report;

   b. NCDR Data Elements and reports from CathPCI and ACTION registries;

   c. Primary and total PCI volume/year for each cardiologist treating EMS transported STEMI patients;

   d. Total time and number of Cardiac Catheterizations and episodes per year that the catheterization lab was unable to perform.

4. Must attend Solano County STEMI Review Meetings

D. Solano County STEMI Review Meeting Participation

1. Present and discuss data on system performance for entire system and each SRC.

2. Discuss cases that failed to meet door to balloon time of 90 minutes, cases that did not follow protocols, and other outlying cases.

3. Discuss EMS performance on STEMI cases to identify opportunities for improvement and to recognize outstanding performance.

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Participation in American College of Cardiology National Cardiovascular Data Registry (NCDR®)	NCDR® Aggregate Data to be reported: Quarterly (raw) with adjusted data from NCDR® when available to include all primary PCI interventions (EMS and non-EMS) • Number of patients with primary PCI intervention • Median door-to-intervention interval (minutes) Percentage and numerator/denominator of patient counts for the following: • STEMI Mortality • PCI Mortality • Procedural Success • Vascular Complications • ASA upon arrival within 24 hours • Beta-blockers upon arrival within 24 hours • ASA on discharge • Beta-Blockers on discharge • ACE Inhibitors or ARM in patients with Ejection Fraction <40% on discharge.	Data shall be submitted within 3 months of completion of calendar quarter. (Data elements may evolve over time.)
Participation in Solano County EMS Data Collection	EMS Data Elements: • STEMI Alert called by EMS (Yes/No/Unknown) • ED ECG STEMI (Yes/No/Unknown) • ED Arrival Time and Date • Intervention Done (PCI, thrombolysis, or no intervention) • Intervention Time and Date • STEMI patient transfers to another STEMI Receiving Center and reason for transfer	Data shall be submitted within 10 days of date of patient arrival. (Data elements may evolve over time.)
Quarterly STEMI QI Committee Data Reports	EMS Data Report to include: • Number of STEMI Alerts • Number of confirmed STEMIs (of those with alert) • Number of Interventions and Type (PCI or thrombolysis) • Door-to-Intervention Interval (median) by type (in minutes) • Percentile of Door-to-Intervention 90 minutes or less (PCI), 30 minutes or less (thrombolysis) – (numerator and denominator of both categories) • Number of times and reason cath lab was unavailable, (e.g. another patient on table, cardiologist unavailably, cath lab staff unavailable, or equipment failure.) • Number of times a STEMI Patient had to be transferred from one STEMI Receiving Facility to another	Data shall be submitted within 3 months of completion of calendar quarter. (Reports may evolve based on QI findings and data element change.)
Annual STEMI QI Report	Annual STEMI QI Report • EMS Data Report • NCDR Data Elements • Cardiologist Primary and Total PCI volume/year for those treating EMS-transported patients • Total time and number of episodes per year that catheterization lab not able to function.	Data shall be submitted within 3 months of completion of calendar year.

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POLICY MEMORANDUM 6610

Implementation Date: 12/01/13
Release Date: 10/14/13

REVIEWED/APPROVED BY:

AARON BAIR, MD., MSc, EMS AGENCY MEDICAL DIRECTOR

TED SELBY, EMS AGENCY ADMINISTRATOR

SUBJECT: End Tidal Carbon Dioxide (ET CO_2) Monitoring

AUTHORITY:CALIFORNIA HEALTH & SAFETY CODE, DIVISION 2.5, 1797.220 & 1797.221

PURPOSE: It is the purpose of this policy to ensure proper use of End Tidal CO_2 ($ETCO_2$) Monitor devices, to provide optimal care to all intubated patients utilizing end tidal CO_2 monitoring, to confirm correct placement of the endotracheal tube, and/or to measure the adequacy of ventilation and perfusion of patients utilizing ventilatory assist devices.

Monitoring with waveform capnography will allows providers to closely watch the patient's condition and assess treatment.

Manufacturer specific guidelines should be followed.

I. Definitions

A. End Tidal CO_2 Capnography – The process of continuously recording the level of carbon dioxide in expired air. The process is used to monitor critically ill patients. The data are typically recorded automatically on a strip of graph paper and numerically. The normal value for $ETCO_2$ is 35 – 45 mm Hg. This process is more reliable in determining correct placement of an endotracheal tube.
B. Capnograph – an instrument used to produce a capnogram, a tracing that shows the proportion of carbon dioxide in a volume of gas.
C. Capnometry - Is the measurement of CO_2 in a volume of gas.
D. Capnometer - a device for monitoring the end-tidal partial pressure of carbon dioxide. Generally using color change to indicate the presence of carbon dioxide. This type of device is not as accurate as End Tidal CO_2 monitoring for endotracheal tube verification.

II. Training

Each Advanced Life Support (ALS) Provider shall provide paramedics training on the use of $ETCO_2$ Monitoring, including but not limited to, use, trouble shooting, and waveform evaluation.

A. Each ALS Provider shall provide training approved by the Agency's Medical Director and Solano County EMS Agency on $ETCO_2$, based on the manufacturer's recommendations.
B. Each paramedic must be trained in the use of $ETCO_2$ before they are authorized to use the device.

III. Procedure

A. Attach the End-Tidal CO_2 detector to the Blind Insertion Airway Device (BIAD) or the Endotracheal Tube (ET). Capnography is the preferred method of verifying correct placement of the endotracheal tube.
B. Note the CO_2 level and waveform changes. Levels and waveforms are to be documented on the PCR. A copy of all waveforms will be attached to PCRs..
C. Capnography will remain in place and be monitored throughout the duration of the call, or until the patient is transferred to a higher level of care.
D. Any loss of CO_2 detection or waveform will be documented.
E. In all patients with a pulse an $ETCO_2$ > 20 is anticipated. In the post-resuscitation patient no effort should be made to lower $ETCO_2$ by modification of the ventilator rate. In post-resuscitation patients without evidence of ongoing, severe bronchospasm, the ventilator rate should never be < 6 breaths per minute.
F. Any significant movement, emesis, or change in clinical condition shall be reassessed and documented in the PCR. If at any time the capnography indicates that the tube is not in the trachea the airway must be removed and the patient re-intubated.

IV. Quality Improvement:

A. The ALS Provider will audit the use of $ETCO_2$ on a monthly basis to ensure the procedure is followed. This information will be reported during the quarterly PCC/CQI meeting.

   a. This report will include, but not be limited to, the number of times the $ETCO_2$ was used, number of total calls, percentage of use, failures, etc.

B. ALS Providers will provide an annual record of paramedics trained in the use of $ETCO_2$ by February 28 of each year..

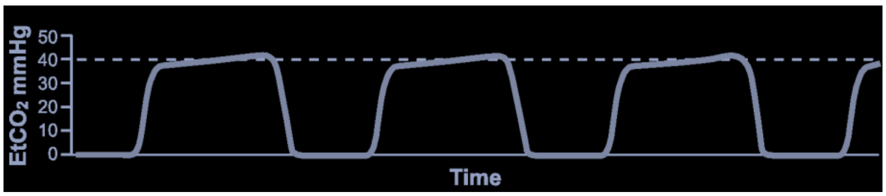
VI. Documentation.

A. Document $ETCO_2$ numeric values and attach a copy of the waveform to the Patient Care Report (PCR).

B. Capnography should be monitored in one minute intervals and each time the patient is moved. All numeric values will be documented on the PCR.

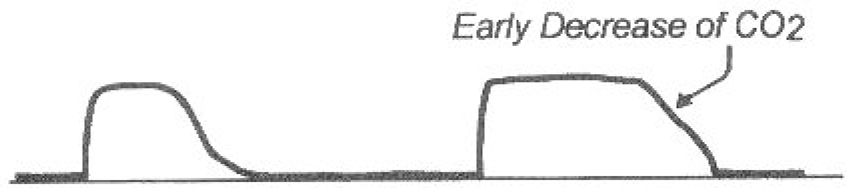
A. Examples of waveforms and explanations.

Intubated Patient.

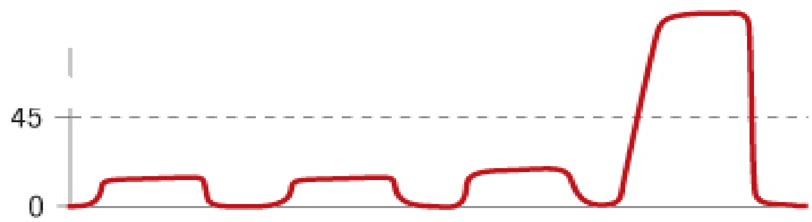


Esophageal Intubation

Cuff Leak



Return of Spontaneous Circulation (ROSC)



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Solano County Health & Social Services Department

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Ted Selby
EMS Agency Administrator

POLICY MEMORANDUM 6611

Implementation Date: December 1, 2016
Review Date: December 1, 2018

REVIEWED/APPROVED BY:

BRYN E. MUMMA, MD, MAS, EMS AGENCY MEDICAL DIRECTOR

TED SELBY, EMS AGENCY ADMINISTRATOR

SUBJECT: SPINAL MOTION RESTRICTION (SMR)

AUTHORITY: California Health & Safety Code Division 2.5, Sections 1797.170, 1797.171, 1797.172, 1797.204, 1797.220
California Code of Regulations Title 22, Division 9, Sections 100063, 100106, 100147

PURPOSE:

To establish criteria as to when Spinal Motion Restriction (SMR) should be performed to reduce unnecessary field immobilizations.

I. DEFINITIONS

A. Spinal Motion Restriction (SMR) – Term used to describe the procedure restrict the movement of the spine for patients with possible unstable spinal injuries with devices such as, but not limited to, full spine boards, cervical collars, head blocks, Kendrick Extrication Device (KED), or full body vacuum splints.

B. Spinal Injury Assessment – Spinal Injury Assessment will include the following elements:

1. Palpation of the entire vertebral column to assess for tenderness or
1. deformities;
2. Wrist and finger extension bilaterally to assess motor function;
3. Plantar flexion (both feet);
4. Dorsiflexion (both feet);
5. Assessing for any abnormal sensations to extremities or loss of sensation to extremities.

Spinal Injury Assessment will be performed on all trauma patients prior to applying/not applying SMR and be documented in the patient care report.

II. INDICATIONS/CONTRAINDICATIONS

A. SMR is indicated in pre-hospital trauma patients who have the potential for spinal injury and have at least one of the following criteria:

1. Altered Mental Status from baseline.
2. Evidence of intoxication.
3. A distracting injury.
4. Any neurological deficit (i.e. paralysis or numbness or tingling in extremities)
5. Midline Spinal pain or tenderness with palpation.

B. SMR is **NOT** indicated and should **NOT** be applied for patients of penetrating trauma to the head, neck or torso unless any of the following are present:

1. Any neurological deficit.
2. Priapism.
3. Neurogenic shock.
4. Visible deformity of the spine.
2. Significant secondary blunt MOI.

C. SMR may be omitted if all assessment criteria are assessed and are normal.

D. For patients experiencing localized cervical spine pain or tenderness **ONLY**, a cervical immobilization device may be used without immobilizing the rest of the spine.

III. SMR METHODS

A. If a patient experiences negative effects of the SMR methods used, alternative methods may be implemented. Alternate SMR methods may include, but not limited to the following:

1. Patient positioning (i.e. lateral, semi-fowlers, fowlers).
2. Pillows.
3. Vacuum splints or mattresses.
4. Car seats.
5. Kendrick Extrication Device (KED).
6. Backboards and head immobilizers.

IV. PROCEDURE

A. Provide manual stabilization restricting gross movement. Alert and cooperative patients may be allowed to limit motion, if appropriate with or without a cervical collar.

B. Apply cervical collar, if clinically indicated.

C. Extricate the patient limiting flexion, extension, rotation, and distraction of the spine.

D. If the decision to use SMR has been made, consider the following:

1. Keeping with the goals of restricting gross movement of the spine and preventing increased pain and discomfort, self-extrication by the patient is allowable.
2. Pull sheets, other flexible devices, scoops or scoop-like devices may be utilized. Hard backboards should have limited utilization.

E. Place the patient in the best position to protect the airway.

F. Regularly reassess motor and sensory function as indicated in the Spinal Injury Assessment.

V. SPECIAL CONDISERATIONS

A. Low risk factors where SMR may be omitted after a complete Spinal Injury Assessment is performed include:

1. Simple rear-end Motor Vehicle Collisions (MVC).
2. Patient ambulatory on scene at any time.
3. No neck pain on scene.
4. Absence of midline cervical tenderness.

B. High risk factors where SMR should be strongly considered include:

1. Patient age greater than 65 years old.
2. Meets Trauma Triage Criteria for mechanism of injury as outlined in Policy 6105.
3. Axial load to the head.
4. Numbness and/or tingling to the extremities.

C. Patients experiencing difficulty breathing in SMR have been found that respiratory function is limited by 17% with the greatest effect experienced by geriatric and pediatric patients restricted to a hard backboard. For patients experiencing difficulty breathing the head of the backboard may be raised slightly to accommodate relief of the respiratory complaint.

D. Pediatric patients in car seats will be immobilized as follows:

1. For rear facing car seats, the patient may be immobilized and extricated in the car seat. The patient may remain in the car seat if the immobilization is secure and as patient condition allows.
2. For front facing car seats with a high back, the patient may be immobilized and extricated in the car seat. Once removed from the vehicle, the patient should be placed in SMR.
3. For booster seats, the patient will be extricated and immobilized following standard SMR procedures.

If the decision is made to apply SMR to patient in a car seat, ensure that a proper Spinal Injury Assessment of the patient's posterior is performed.

E. Football helmets should not be removed; they should be immobilized in place. Be sure to pad around the patient's helmet, neck and shoulders to fill any gaps and maintain inline spinal motion restriction.

1. Football helmets should only be removed if the helmet is interfering with procedures used in securing and maintaining the airway.
2. All other types of helmets can be removed under this policy.

F. Pregnant patients in the third trimester should be transported in the left-lateral position to prevent supine hypotension.

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GERALD HUBER
Director

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POLICY MEMORANDUM 6612

Implementation Date: August 26, 2023
Review Date: August 1, 2025

REVIEWED/APPROVED BY:

PRANAV SHETTY, MD, MPH, EMS AGENCY MEDICAL DIRECTOR

BENJAMIN GAMMON, EMT-P, EMS AGENCY ADMINISTRATOR

SUBJECT: USE OF CARDIAC MONITOR AS AN ASSESEMENT TOOL

AUTHORITY: Health and Safety Code, Division 2.5, Sections 1797.200, 1797.220,

I. PURPOSE:

To establish procedures to allow for discontinuing a cardiac monitor to facilitate transfer of care to Basic Life Support (BLS) units to transport non-emergent, stable patients.

II. CARDIAC MONITOR USE AS AN ASSESSMENT TOOL IN DERTERMINATION OF BLS TRANSPORT

A. Advanced Life Support (ALS) personnel may use a cardiac monitor as an assessment tool to in the evaluation of any patient with a medical or trauma complaint.

B. If it is determined by the ALS personnel that the patient is stable with non-life-threatening complaints and does not require any ALS treatment per protocol, the cardiac monitor may be discontinued to facilitate transfer of care to a BLS unit for transport.

1. If any other ALS treatment has been initiated, the cardiac monitor cannot be discontinued, unless allowed by protocol or base hospital order, and the patient cannot be transferred to a BLS unit for transport.

III. QUALITY ASSURANCE/QUALITY IMPROVEMENT

A. Any discontinuation of a cardiac monitor as described in Section II of this policy will be 100% audited by the transport provider to ensure patient safety and adherence to existing policy and protocol.

B. If any abuse of this policy is found during an audit, the provider must file a Field Advisory Report (FAR) as described in Policy 2305.

1. All FAR submissions on the abuse of this policy may result in an investigation by the EMT's certification entity or investigation by the California State EMS Authority for Paramedics.
